

ONLINE NIGHT 2 · 5:30 PM – 8:00 PM CST

Labor, Birth & When Things Get Complex

Mary's Hands Network

ICEA Certified Birth Doula

Hybrid Training

Learning Objectives

Four stages, three phases — get the language right

How labor hormones respond to your presence

The Six P's of labor progress

Induction & augmentation — ARRIVE trial

Doula scope with medicated clients & complications

Doula role when birth becomes loss

SECTION ONE

Stages & Phases of Labor

*Birth doesn't unfold in neat boxes — it flows like a river.
Our job is to help families navigate it.*

Four Stages. Three Phases. One Common Confusion.

This distinction is tested on the ICEA certification exam — and it matters in the clinical setting.

Four Stages of Birth

Stage 1: Labor (0–10 cm)

Stage 2: Pushing & birth

Stage 3: Placenta delivery

Stage 4: Recovery — the Golden Hour

Three Phases of Stage 1

- Early/Latent phase (roughly 0–6 cm)
- Active phase (roughly 6–8 cm)
- Transition phase (8–10 cm)

Stages have distinct start/end points. Clinical language.

Phases flow into each other gradually — like a dimmer switch, not on/off.

Early / Latent Labor

The longest phase — and often the hardest to support because it can last hours or days.

What's happening

Cervix softening, thinning,
opening

Dilation: 0–6 cm

Contractions: 30–45 sec, 5–20
min apart

Manageable at home — rest, eat,
hydrate

Nesting burst, chatty, excited

Duration: 8–24 hours for first
babies

Doula Script

“You’re in early labor — this is real and it’s happening. Right now, the best thing you can do is rest, eat something light, and let your body do its work. Don’t time every contraction yet. Try to sleep if it’s nighttime. Call me when they’re regular and strong enough that you need to focus through them.”

Doula task: Conserve energy. Prevent early hospital arrival.
Normalize the wait.

Prodromal Labor

Real contractions. Real pain. No measurable cervical progress — yet. One of the hardest things to support.

What It Is

Regular painful contractions — no cervical change

Hospital may send her home (demoralizing)

Baby may need to shift position

Stress can be blocking active labor

Doula Script

“I know this is so frustrating. These contractions are real and they hurt, and you’ve been doing this work for hours. What’s happening is your body is preparing — softening your cervix, helping your baby get into position. This isn’t wasted effort. Every contraction is doing something important, even if we can’t measure it in centimeters yet. Let’s focus on rest between contractions and conserving your energy for when things really pick up.”

Active Labor — When Things Get Real

She can no longer talk through contractions. This is when she needs you most.

What's happening

Dilation: 6–8 cm

Contractions every 3–5 min, 45–90 sec

Talking stops — coping begins

Hospital typically admits now

Average 1–2 cm/hour (varies)

Client turns inward — protect her focus

Clinical Note — The ACOG 2014 Update

Active labor does not reliably begin until 6 cm. Before 2014, the threshold was 4 cm. This change was made to reduce unnecessary “failure to progress” diagnoses and cesareans. When a provider says she’s “not in active labor,” ask: is the cervix changing at all?

Your doula role

- Continuous presence — don’t leave the room
- Rhythm: breathe with her, move with her

Transition — The Peak

Shortest phase. Most intense. Your anchor matters more here than anywhere else.

What it looks like

Dilation: 8–10 cm

Contractions every 1–3 min, 60–90 sec

Shaking, nausea, vomiting are normal

"Labor land" — altered consciousness

Duration: 20–90 minutes

"I can't do this" = almost there

Doula Scripts — The Transition Anchor

"You are doing it. Right now. This is the hardest part and you are already in it. Look at me. Breathe with me. One contraction at a time. You don't have to get through all of them. Just this one."

When she says "I can't do this":

"I know. I hear you. And you already are doing it. This is transition — which means you are almost there. Your baby is almost here."

Stage 2 — Pushing & Birth

10 cm to baby in arms. A rush of relief — or the hardest work yet.

What's happening

Full dilation (10 cm) → baby born

Duration: 30 min – 2 hrs (first babies)

Urge to push = intense pressure

Fetal ejection reflex may occur

"Ring of fire" as head crowns

Upright, hands-knees, side-lying all valid

Your doula role

- Hold her leg or support position if needed
- Mirror breathing if panic-pushing

Doula Script — At Crowning

"You're almost there. Your baby's head is right here. Slow breath now — just breathe your baby out. You're going to meet them in the next few pushes."

Stage 3 — The Placenta

The only organ the body grows for nine months — and then releases.

What's happening

From birth to placenta delivery

Duration: 5–30 minutes

Uterus contracts to expel placenta

Signs: blood gush, cord lengthens

Two types: physiological or active

Retained >30 min → alert provider

Clinical Note — Delayed Cord Clamping

ACOG (2020) and WHO recommend waiting at least 30–60 seconds before clamping — ideally until the cord stops pulsing (3–5 min). This transfers up to 30% more blood volume to the baby and reduces neonatal anemia risk. Advocate for this in the birth plan.

Your doula role

- Client is focused on baby — you watch for heavy bleeding
- Remind her: a few more pushes, almost done

Stage 4 — The Golden Hour

You are the guardian of this time. Your job is to protect it.

Why it matters

First 1–2 hours postpartum — bonding window

Oxytocin flood from skin-to-skin

Prolactin spike → milk production

Endorphins → "birth high"

Stabilizes baby temp, HR, blood sugar

Don't interrupt this physiology

Doula Scripts — Protecting the Hour

To staff: "Is there anything that absolutely has to happen right now, or can we let them have some uninterrupted time first?"

To partner: "Tell her how amazing she is. She just did something extraordinary."

Routine procedures that CAN wait

To family: "Visitors can wait. This hour is theirs."

- Weight & measurements: 1–2 hours

- First bath: up to 24 hours (vernix is protective)

Doula Task Mapping

In the chat, drop your answer: You've just arrived at the hospital with your client. She's 5 cm. Thirty minutes later she's still 5 cm. Two hours after that she's 7 cm and shaking. What's your doula role at each of those three moments — and what changes?

Discuss with your group:

- What changes about your role from early labor through transition?
- What does “continuous support” actually look like hour by hour?
- When is silence more supportive than talking?

SECTION TWO

The Hormonal Symphony of Labor

Labor doesn't start in the uterus. It starts in the brain.

The body releases hormones only when it feels safe, supported, and undisturbed.

The Labor Hormones — An Overview

Your calm presence isn't just emotional support — it is hormonal support.



Oxytocin

The Love Hormone

Triggers and sustains contractions. Released by touch, warmth, and safety. Pitocin is its synthetic form.



Endorphins

Natural Pain Relief

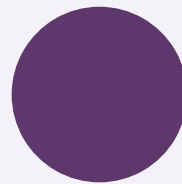
Rise with contraction intensity. Create “labor land.” Blocked when adrenaline rises.



Adrenaline

The Fear Response

Slows labor when threat is perceived. Cold, bright lights, fear trigger it. Your job: keep it low.



Prolactin

The Mothering Hormone

Spikes in the Golden Hour. Prepares for milk production and shapes the transition into parenthood.

Oxytocin — “The Love Hormone”

Your calm, warm, reassuring presence is not just emotional support — it is hormonal support.

Triggers and sustains contractions

Released by touch, warmth, safety

Pitocin is synthetic — same molecule, no feedback

Drops when afraid, observed, unsafe

Floods at skin-to-skin → bonding + bleeding control

Clinical Note — What This Means for Your Role

The 2017 Cochrane Review found continuous doula support leads to shorter labors and fewer cesareans. One mechanism: doulas create the psychological safety that keeps oxytocin flowing. Reducing fear → lowering adrenaline → allowing oxytocin to work. This is the physiology of your presence.

Endorphins — The Body's Own Pain Relief

"Labor land" is endorphins doing exactly what they're supposed to do.

Natural opioids made by the brain

Rise progressively with contractions

Create "labor land" dissociation

Drive the "birth high" after birth

Suppressed by adrenaline (fear, cold, lights)

Epidural blocks them — emotional support still essential

Adrenaline — The Fear Response

A late-labor surge of adrenaline is normal and gives energy to push. Chronic fear-driven elevation stalls labor.

What it does

Body's threat-response hormone

Triggers fight-or-flight — blood leaves uterus

Directly suppresses oxytocin

Blocks endorphins → pain increases

Late-labor surge is normal — gives push energy

Chronic fear elevation stalls labor

What Triggers Adrenaline in Labor

- Bright lights and cold rooms
- Loud voices, too many people, lack of privacy
- Fear of birth, of pain, or of the unknown
- Prior birth trauma or medical trauma

Do not intervene: Dim the lights. Lower your voice.

Reduce foot traffic. Create safety. Every environmental adjustment you make is a hormonal adjustment.

Being observed or interrupted while coping

Feeling that something is

Prolactin — The Mothering Hormone

Prolactin thrives in quiet, undisturbed bonding time. Protect it.

Initiates and sustains milk production

Spikes dramatically in the Golden Hour

Stimulated by skin-to-skin + nipple contact

Shapes attentiveness to baby's cues

Disrupted by early formula supplementation

Thrives in dim, quiet, undisturbed conditions

Clinical Note — Your Role in the Golden Hour

Every routine procedure you delay, every visitor you redirect, every quiet moment you protect is directly supporting prolactin. You are not being difficult. You are protecting physiology.

BREAK

6:20 PM — 6:30 PM · 10 minutes

Step away, stretch, hydrate, reset.

Back at 6:30 PM — Six P's and Induction up next.

SECTION THREE

The Six P's of Labor

Labor progress depends on six factors working together.

Think of them as gears in a machine — if one is off, all of them feel it.

The Six P's — What Drives Labor Progress

Your continuous presence addresses all six simultaneously.

Powers

Strength, frequency & coordination of contractions. Involuntary in Stage 1; voluntary pushing added in Stage 2.

Passage

The bony pelvis and soft tissues (cervix, vagina). Shape, size, and opening all matter.

Passenger

The baby: size, position, and how well the head can flex and rotate to navigate the pelvis.

Position

The birthing person's position during labor. Upright and mobile positions optimize all other P's.

Psyche

Emotional state. Fear slows labor. Safety speeds it. This P responds most directly to your presence.

Provider

The support team dynamic. Communication, trust, and shared decision-making all affect progress.

Powers — The Contractions That Drive Labor

You cannot make contractions happen. You can only create the conditions that allow them to work.

Stage 1: Cervical dilation

Involuntary — uterus contracts on its own

Measured by frequency, duration, intensity

Goal: efface and dilate 0 → 10 cm

Adequate: every 2–3 min, 60–90 sec

Stage 2: Pushing

- Voluntary bearing-down effort added to involuntary contractions
- Fetal ejection reflex: body pushes spontaneously — support

Clinical Note — Inadequate Powers

When contractions are too weak, too far apart, or poorly coordinated, the cervix may not dilate despite hours of effort. This is one reason augmentation with Pitocin is recommended. As a doula, you may notice this before anyone says anything — contractions you can barely see from across the room, long gaps, no reported progress.

Doula role

Optimize the environment for oxytocin. Position changes to encourage descent. Emotional support to keep adrenaline low.

Passage & Passenger — Pelvis & Baby

The passage is the road. The passenger is who's traveling. Both have to cooperate.

Passage — The Bony Pelvis

Pelvis has multiple diameters to navigate

Gynecoid shape is most favorable

Soft tissues must soften and stretch

Position changes optimize entry angle

Squatting opens outlet up to 28%

Passenger — The Baby

- Baby's size, position, and ability to flex and rotate affect progress
- Ideal position: occiput anterior (OA) — baby faces mother's back
- Baby must flex chin to chest, rotate, and extend to emerge
- Cardinal movements: engagement, descent, flexion, internal rotation, extension.

Position & Psyche — The Two You Control Most

Of all six P's, position and psyche respond most directly to what a doula does.

Position — The Birthing Person

The most modifiable P — change every 20–30 min

Upright uses gravity, opens pelvis

Hands-knees relieves back labor

Side-lying conserves energy (epidural-friendly)

Walking, swaying, stairs all encourage descent

Movement matters more than any one position

Psyche — The Emotional State

– Fear, anxiety, and trauma history directly affect hormone balance

– The fear-tension-pain cycle:
fear → tension → more pain →

Doula Script — Psyche Check-In

“I’m right here. You’re doing great. Tell me what you need right now.”

Provider — The Team Dynamic

The quality of the birth team relationship is a clinical variable.

How Provider affects labor

Trusted team → less fear → better psyche

Clear communication → adrenaline drops

Rushed/dismissive care → cortisol up

Supportive nurse is a force multiplier

You work alongside the provider — bridge, never undermine

Cochrane Review — All Six P's Connected

The 2017 Cochrane Review: continuous support leads to 25% fewer cesareans, 10% less pain medication use, and labors averaging 41 minutes shorter. The mechanism: doulas address all Six P's simultaneously.

How doulas address all Six P's

- Powers: optimize the hormonal environment
- Passage: encourage position changes and relaxation

SECTION FOUR

Induction & Augmentation

Induction starts labor. Augmentation speeds it up.

Your job is to help families make informed decisions — not to make the decision for them.

Induction vs. Augmentation

Think breast augmentation: you already have some, now you have more. Labor augmentation is the same idea.

Induction — Starting Labor

Body is not yet in labor

Methods to GET labor started

Zero contractions → active labor

Requires cervical readiness (Bishop)

May need ripening before Pitocin works

Augmentation — Strengthening Labor

- Labor has already started
- Contractions are too weak, too far apart, or stalled
- Methods used to strengthen or speed up what's there
- Most common: Pitocin drip or AROM
- Does NOT mean something is wrong — often a normal part of labor

Why Induction Is Recommended

Medical and elective inductions require very different preparation conversations.

Medical Indications

Past 41–42 weeks (post-dates)
Water broke, labor hasn't started (PROM)
Preeclampsia or dangerous BP
Baby not growing adequately (IUGR)
Gestational diabetes concerns
Other health concerns

Elective Induction

- Scheduling or partner availability
- Distance from hospital
- History of fast labors
- Not recommended before 39

Clinical Note — The Bishop Score

Providers assess cervical readiness with the Bishop Score (dilation, effacement, consistency, position, station). Score 8+ = favorable for induction. Lower = cervical ripening first,

and induction may take 12–24 hours before active labor

begins.

Induction Methods — At a Glance

Cervical ripening comes first if the cervix isn't ready. Pitocin cannot do its job on an unripe cervix.

Method	What It Does	Your Doula Role
Cervidil / Cytotec	Prostaglandins soften and thin the cervix. Cervidil = insert (removable). Cytotec = pill (not removable). Usually given overnight. Causes cramping.	Normalize the long wait. Keep her comfortable. Encourage rest — this phase can take 12+ hours before Pitocin even starts.
Foley Bulb	Balloon catheter inflated on both sides of the cervix to manually press it open. No medication. Falls out around 3–4 cm dilation.	Warn her it feels like strong pressure. Help her breathe through insertion. Once placed she can usually walk.
Pitocin (IV)	Synthetic oxytocin via IV drip, starting low and increasing until effective contractions every 2–3 min. Requires continuous fetal monitoring.	Pitocin contractions can be stronger than natural ones. All comfort measures still apply. Use the bed and peanut ball strategically.

The ARRIVE Trial — What the Evidence Actually Says

Clients will ask you about this. Know it cold — then help them ask their own questions.

What the ARRIVE Trial Found (2018)

A large randomized controlled trial comparing elective induction at 39 weeks to expectant management in low-risk pregnancies. Key finding: induction at 39 weeks did NOT increase cesarean rates — and was associated with slightly lower rates of some perinatal complications. This directly challenged the long-held belief that induction automatically leads to more cesareans.

What this means

- Many providers now offer 39-week elective induction
- Not automatically harmful — evidence doesn't support that
- Individual risk, preferences, and context still matter

Doula Language — When Asked About ARRIVE

*“Have you asked your provider about the benefits and risks for your specific situation?
Would it help to write down your questions before that appointment?”*

BRAIN Framework Practice

The Scenario:

A client texts you at 40+1 weeks: “My doctor wants to induce next week. Should I do it? I was really hoping to go into labor on my own.”

In the chat — type your word-for-word response. Then discuss:

B — Benefits of induction now? **R** — Risks?

A — Alternatives? **I** — What does her intuition say?

Scope reminder: Help her think through the decision using BRAIN — never steer it. You may not change the intervention, but you can change her experience of making the decision.

SECTION FIVE

Pain Relief in Labor

*There is no wrong choice. There is only the informed choice
that is right for this person, in this moment, in this labor.*

The Spectrum of Pain Relief

Pain relief is not binary. It exists on a spectrum — and your doula role is constant across all of it.

Non-Pharmacological

Breathing & vocalization
Hydrotherapy (shower/tub)
Counter-pressure & massage
Movement & position changes
Heat & cold therapy
Acupressure

Nitrous Oxide

- Inhaled at contraction onset
- Reduces pain perception — doesn't eliminate it
- Client holds the mask (self-administered)
- Wears off immediately between contractions
- Not available at all facilities
- Doula: coach timing, monitor for dizziness

IV Opioids

- Fentanyl or morphine via IV
- Takes the edge off without full numbness
- Causes drowsiness — allows rest
- Wears off in 1–4 hours
- Not given when pushing is close
- Doula: your presence more important than ever

Epidural / Spinal

- Most effective option available
- Complete numbness below waist
- Placed by anesthesiologist
- Requires continuous monitoring
- Client cannot move independently
- Doula role SHIFTS — does not stop

The Epidural — What It Is & How You Show Up

Your work doesn't stop when the epidural goes in. It shifts.

What happens

Catheter in epidural space

Continuous medication — numb below waist

15–20 min to full effect

Stay still during placement

Continuous fetal monitoring + IV

BP may drop initially — nurse monitors

How your role shifts

- Physical comfort → positioning becomes your primary work
- Peanut ball + position changes every 20–30 min
- Emotional support matters more — not less — with reduced sensation
- Help client stay connected to her baby and her body

Safety — Moving an Epiduralized Client

- Always use two people to reposition — legs may be numb
- Lift completely off the bed, never drag across sheets

Spinal Block & Combined Spinal-Epidural (CSE)

Spinals are fast, complete, and temporary. Most often used for cesarean birth.

Spinal Block

- Single injection into the spinal fluid — not a catheter
- Takes effect in 1–5 minutes, complete numbness below waist
- Lasts 1.5–3 hours depending on medication
- Standard for planned cesarean births
- Client must remain still during injection
- Sensation returns gradually from top down

Combined Spinal-Epidural (CSE)

- Spinal for fast relief + epidural catheter left in place
- Fast onset + ability to extend or top up
- Common for long labors where epidural is desired
- Sometimes called a 'walking epidural' — mobility varies

Clinical Note — Supporting Placement

During placement she must hold still through contractions. Position her sitting on the bed edge, feet on a stool, chin to chest. Stand in front, hold her hands or forearms. Say:

"Breathe with me. You just need to hold still for the next two minutes. I've got you."

Doula Dos and Donts in Medicated Labor

An epiduralized client needs you just as much. Your role adapts — it doesn't disappear.

DO	DON'T
Reposition with the nurse every 20–30 min (peanut ball, side-lying, semi-reclined)	Leave her alone because she's comfortable now
Use peanut ball to open the pelvis and encourage baby's descent	Skip positioning because she can't feel it — she still needs it
Provide continuous emotional support and narrate what's happening	Assume pain relief = emotional relief — anxiety often surfaces now
Help her stay connected: hand on belly, talk to baby, partner time	Check out and scroll your phone while she rests
Coach partner: how to help, what to say, how to be present	Take over the partner's role entirely
Advocate for laboring down if she feels no urge (delayed pushing)	Push her before the urge arrives without the nurse's input

The Peanut Ball — Use It Every Time

The peanut ball creates the pelvic opening that walking and squatting would provide. Use it every time.

What it does

Peanut-shaped ball between knees side-lying

Holds pelvis open and asymmetrical

Mimics squatting & walking

Reduces labor length and cesarean rate

Rotate sides every 20–30 minutes

How to use it

- Smaller end between the knees, larger end behind the hips
- Top knee rests over the ball, not hanging off it

Clinical Note — Peanut Ball & Outcomes

A 2020 systematic review found peanut ball use reduced Stage 1 labor by 90 minutes on average and lowered cesarean rates in epiduralized clients. One of the most impactful tools in your kit — and it costs nothing.

BREAK

7:20 PM — 7:30 PM · 10 minutes

Step away, stretch, hydrate, reset.

Back at 7:30 PM — When Birth Needs Help: complications & loss.

SECTION SIX

When Birth Needs Help

Your job isn't to prevent all interventions.

It's to help families navigate them with dignity and agency.

Normal Variations vs. True Complications

Not everything that deviates from textbook labor is a complication. Some are variations.

Normal Variations

Common — often resolve with position changes, patience, and support:

- Posterior positioning (sunny-side up)
- Nuchal cord (cord around the neck)
- Slow early labor or prolonged latent phase
- Brief fetal heart rate decelerations that resolve quickly
- Asynclitic presentation (head tilted to one side)

True Complications

Require provider involvement — sometimes urgently:

- Fetal distress — persistent concerning heart rate patterns
- Maternal hemorrhage — excessive bleeding
- Prolapsed cord — cord slips through cervix before baby
- Placental abruption — placenta separates prematurely
- Shoulder dystocia — shoulders stuck after head delivers
- Uterine rupture — rare but catastrophic

OP Position — Back Labor

15-30% of babies start labor posterior. Most rotate. Your positioning work matters.

What's happening

Occiput posterior — baby faces front
Hard skull presses on sacrum
Intense, unrelenting back pain
Labor often longer (larger diameter)
Only 5–8% remain posterior at delivery

Positions that encourage rotation

- Hands-and-knees — opens back of pelvis, best for 30+ min
- Side-lying on the side OPPOSITE baby's back
- Lunge (one foot on a chair) to

Doula Script — Back Labor

"This pain is real and it's intense. Baby is in a position that's making labor harder. Let's try some positions to help baby rotate. You are

doing exactly the right things."

against rotation

Prolonged Labor — When Exhaustion Becomes the Risk

Prolonged labor is defined as >20 hrs for first-time moms, >14 hrs for subsequent births.

Why it matters

Uterus exhausts after hours of work

Maternal exhaustion → hemorrhage risk

Infection risk rises after ROM

Baby tolerates prolonged labor less well

Intervention can be mercy, not failure

Clinical Note — When Intervention Becomes Necessary

A client who has labored for 40+ hours without adequate rest, who cannot eat or drink, and whose cervix has stopped progressing is not “failing.” Her body is working beyond its sustainable limit. Epidural for rest, Pitocin augmentation, or cesarean birth may be the kindest, ~~most evidence-based choice available to her.~~

Doula Script — When Intervention Is Recommended

“Your body has worked so hard for so long. Choosing to rest or accept support right now is not giving up. It’s wisdom. You are still doing this. You are still the one having this baby.”

Shoulder Dystocia — Get Out of the Way

0.5–1.5% of vaginal births. Rare, but not so rare that you won't encounter it.

What it is

Head delivers, shoulders stuck behind pubic bone

Cord compressed — baby cannot breathe

Must be delivered within 4–5 min

"Turtle sign" — head retracts back

Providers act in seconds

Your role — Four things only

- Move out of the way — providers need space immediately
- Stay calm — your client feels your energy
- Support the partner briefly: "The team has this"
- Stay present but silent until baby is delivered

After Birth — What Families Need

"That was scary. You did nothing wrong. The medical team acted

quickly, and your baby is here.

Cesarean Birth — Planned vs. Emergent

A cesarean birth with dignity and agency is a better outcome than a vaginal birth filled with fear.

Planned Cesarean

- Scheduled in advance — known date and time
- Client arrives calm and prepared
- You can pre-plan your role and her preferences
- Gentle/family-centered C-section may be possible: clear drape, immediate skin-to-skin
- Doula may be allowed in OR at some facilities — confirm in advance
- Support partner coaching before, during, and after

Emergent Cesarean

- Happens fast — client may not have time to process
- You may not be allowed in the OR
- Stay with the partner if you're separated from the client
- Keep partner calm and informed
- When she returns, she may need to process what happened
- Your presence in recovery is critical — don't leave early

VBAC — Vaginal Birth After Cesarean

72–75% success rate. Your role is informed support — not steering the decision.

Key Facts

- 72–75% of VBAC attempts result in vaginal birth · 0.5–1% risk of uterine rupture (rare but serious)
- Best candidates: one prior cesarean, low transverse incision, supportive provider, hospital equipped for emergency
- Laboring naturally is preferred — Pitocin slightly increases uterine rupture risk

VBAC	Elective Repeat Cesarean
72–75% success rate	Known birth date, planned recovery
Shorter recovery if successful	No risk of rupture during birth
Increases chance of future vaginal births	Longer initial recovery
Requires labor and continuous monitoring	Future births likely also cesarean

What Would You Do?

The Scenario:

Your client has been in labor for 34 hours. She planned an unmedicated birth and has been doing beautifully. But for the last 6 hours, her cervix hasn't changed. The provider is now recommending a cesarean. She's sobbing. Her partner looks panicked.

Discuss with your group:

- What do you say to her in this moment — word for word?
- What do you say to the partner?
- If she asks “Should I do it?” — how do you respond without answering the medical question?
- How does your support look different in the OR waiting room vs. in recovery?

SECTION SEVEN

Perinatal Loss— An Introduction

Your doula work doesn't end when hope shatters.

In the moments when words fail, presence becomes your most powerful tool.

Why We Cover This — and What It Includes

Grief is not proportional to the size of the baby or length of pregnancy. Every loss is profound.

Types of perinatal loss

- Miscarriage — before 20 weeks
- Stillbirth — fetal death at 20+ weeks
- Neonatal death — within 28 days
- Life-limiting diagnosis
- TFMR — termination for medical reasons
- Loss of a twin / multiple

Why Doulas Need This Training

- You WILL encounter loss during your career — it is not rare
- Families remember forever how they were treated in these moments
- Your presence, language, and actions either help or cause harm
- You do not need to fix grief — you need to witness it

Language — What Helps vs. What Harms

Families will remember what you said forever. Choose your words with that weight.

SAY THIS	NEVER SAY THIS
<i>I'm so sorry.</i>	Everything happens for a reason.
<i>Your baby is beautiful.</i>	God needed another angel.
<i>There's nothing I can say. But I'm here.</i>	At least you can get pregnant again.
<i>Tell me about [baby's name].</i>	At least you have other children.
<i>It's okay to feel whatever you feel.</i>	I know how you feel.
<i>What do you need right now?</i>	They're in a better place now.

Perinatal Loss: Language

Mary's Hands Network · ICBD Hybrid Training · Night 2

Your Role — Bear Witness. Use the Name. Stay.

You don't need the perfect words. You need to show up and not disappear.

What your role looks like

Use the baby's name — every time

Sit quietly — be okay with silence

Stay even when there's nothing to do

No phone, no clock-watching

Ask before touching

Years later they'll remember you weren't afraid

Memory-making — offer, don't push

— Photos — Now I Lay Me Down to Sleep (free professional photographers)

— Hand and footprints — ink impressions or clay castings

— Physical keepsakes: lock of hair, blanket, hat

— Time — uninterrupted time alone with their baby

— Naming and naming

Doula Self-Care After Loss

You cannot pour from an empty vessel. The 24-hour rule exists to protect you.

The 24-hour protocol

Don't process alone in first 24 hours

Don't reach to family to process YOUR feelings

Journal privately if needed — not social media

Rest, eat, be cared for

Debrief within the week — use MHN monthlies

What Not to Do Right After a Loss Birth

- Don't share the story publicly or on social media, ever
- Don't make the family's loss about your feelings

Carry This Without Being Crushed

You are not required to be okay immediately. Grief is the appropriate response to witnessing grief. What you carry from this work is evidence that you were fully present for a family in the worst moment of their lives.

Take a breath.

This material is heavy. That weight is appropriate.

It means you understand what is at stake.

Hold space for whatever this material brings up for you.

You do not have to perform okayness right now.

This is why you're here.

What to Carry With You From Tonight

01

There are four stages of birth and three phases of Stage 1. Get the language right — it matters in the clinical setting.

02

Labor hormones respond to environment. Dim lights, privacy, warmth, and your calm presence are biological support.

03

Active labor doesn't reliably begin until 6 cm. The ACOG 2014 update exists to reduce unnecessary interventions.

04

Induction is common. Your role is to support informed decision-making using BRAIN — never to steer the decision.

05

An epiduralized client needs you just as much. Peanut ball, positioning, and emotional presence don't stop.

06

When birth becomes loss, you stay. You say the baby's name. You do not look away. You bear witness.

Before In-Person Day 1:

1. Stages of Labor Quiz

Complete and pass the quiz on Canvas (stages, phases, hormone functions, medication identification). You must pass to attend In-Person Day 1. Retakes are available.

2. Resource Guide — Keep Adding

Add at least 3 new entries covering labor support, birth trauma, and perinatal loss resources in your community. Guide must be complete by Day 1.

3. Reflection Journal

Ungraded. In Canvas, respond to: What came up for you during the perinatal loss section? What do you need to prepare yourself to support families through loss?